

Name: فريال على اسماعيل علي
National ID: 23811150102125



تقرير طبي

السيد

تحية طبية وبعد,,

بشأن تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته

تاريخ وساعة الخروج :
PM 01:17 05-07-2026

تاريخ وساعة الدخول :
-- --:-- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد أنه

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Supportive and palliative Treatment

الوقت :

-- --:--

التاريخ :

05-07-2026

الطبيب المعالج :
د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية



فريال

على اسماعيل على

عماره ١١ ش النصر - رابعة العدويه

مدينه نصر اول - القاهره

١٩٣٨/١١/١٥

٢ ٣٨ ١١ ١٥ ٠ ١ ٠ ٢١ ٢٥

115295755



٢٠٢٠/٠٢

٢٣٨١١١٥٠١٠٢١٢٥

ربة منزل



الرملة

مسلمة

أنثى

البطاقة سارية حتى ٢٠٢٧/٠٢/٠٥

١





مستشفى مدينة نصر التخصصي مركز الأشعة والرنين المغناطيسي

أ.د. تامر أحمد كمال
أستاذ الأشعة التشخيصية
جامعة بنها

أ.د. مجدى إبراهيم بسيوني
أستاذ الأشعة التشخيصية
جامعة القاهرة

NAME Feryal ali ismaeil

5 May 2026

ID

176353 5/5/2026 12:19:30 PM

BILATERAL BREAST ULTRASOUND EXAMINATION REVEALED:

The Left Breast:

The Left breast shows left LOQ area of altered parenchyma & calcifications extends from 3 ~ 6 o'clock , with multiple irregular hypoechoic masses with calcifications inside all casting post-acoustic shadowing,

- 4 o'clock (2x1.7 cm , , 4cm fmn , , 2 mm from the skin
- 6 o'clock (11x8 mm , about 1 cm fmn, 3 mm from the skin , 2.2 cm from ind4ex mass
- Smaller mass at 3 o'clock 7.8x6 mm , 2 cm fmn , 2 cm from index mass

Free axillary tail with no evidence of pathologically enlarged sizable lymph nodes
Normal contour of the skin with no nipple retraction.

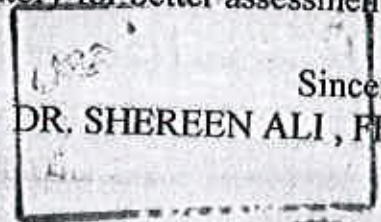
The Right Breast:

- Normal appearance of the areola, skin & subcutaneous tissues with preserved breast contour; apart from nipple retraction.
- Free right axilla with no evidence of enlarged sizable lymph nodes
- Breast parenchyma is sonographically free with no evidence of solid / cystic lesion, dilated ducts or abnormal calcifications.

Conclusion:

- ↓ **LEFT BREAST :** LOQ highly suspicious masses BIRADS IVc; , for histopathological correlation & mammography are mandatory for better assessment
- ↓ **Right breast :** normal (BIRADS I).

Sincerely,
DR. SHEREEN ALI , FEBR



مستشفى مدينة نصر التخصصي ١ ميدان هشام بركات (رابعة سابقاً) مدينة نصر

HOTLINE 19849

تليفون: ٢٤٠ ٣٤٠ ٢٥ / ٦ / ٧ موبايل: ٠١٠٥٠٠٧٨٤٩٩



Patient Name	فريال على اسماعيل على 0	Code Number	YK 297013
Sex/Age	Female 87Y	Patient ID	21844
Registered Date	17/06/2026 08:53:53	Referred By Dr.	Out Patient
Validation Date	25/06/2026 13:03:36	Branch	Gustave Roussy
Reporting Date	6/25/2026, 3:05:13 PM		

breast slide revision

Breast Slide Revision

Clinical Data: Left breast UOQ mass.

Nature of Specimen: Referred block & slides (Tru-cut biopsy).

Gross Examination:

- One referred block labelled by (6998/26). Together with, five referred slides including 1 (H&E) stained slide and 4 immunostained slides for (ER, PR, Her2, Ki67).

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
- * Grade: 2 (differentiation 3, nuclear pleomorphism 2, mitosis 1).
- * Ductal carcinoma in situ: Focal intermediate & high-grade ductal carcinoma in situ (Solid pattern with comedo type necrosis).
- * Angioinvasion: Absent.
- * Perineural invasion: Present.
- * Microcalcification: Absent.
- * Necrosis: Focal.

IPT:

- ER: Negative nuclear reaction in tumor cells (score 0/8).
- PR: Weak nuclear reaction in 2% of tumor cells (score 3/8).
- Her2: Strong complete membrane reaction in more than 10% of tumor cells (score 3+).
- Ki67%: 20%.

Diagnosis:

Left breast mass, Referred block & slides (Tru-cut biopsy):

- Invasive duct carcinoma, Grade 2.
- Focal intermediate & high grade ductal carcinoma in situ (Solid pattern with comedo type necrosis).

IPT:

- ER: Negative (0/8).
- PR: Positive (3/8).

العنوان : 987 كورنيش النيل مصر القديمة - رقم خدمة : 15580 / 0221277647

Patient Name	فريال علي اسماعيل علي 0	Code Number	YK 297013
Sex/Age	Female 87Y	Patient ID	21844
Registered Date	17/06/2026 08:53:53	Referred By Dr.	Out Patient
Validation Date	25/06/2026 13:03:36	Branch	Gustave Roussy
Reporting Date	6/25/2026, 3:05:13 PM		

- Her2: Positive (score 3+).
- Ki67%: 20%.

**** Scoring system: "ER, PR"**

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

**** Scoring system: "Her2"**

Reference: ASCO-CAP Her2 Test Guideline Recommendations. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of

the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature
Dr. Walaa Ibrahim Ghonaim.

Liver Function



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Mizr El Qadima - Cairo, Egypt

العنوان : 987 كورنيش النيل مصر القديمة - رقم خدمة : 15580 / 0221277647



L A B S

Collection Date: 24/05/2026

Review Date: 03/06/2026

Referring Doctor: ا.د سارة زكي

Patient Name: الاستاذة/ فريال علي اسماعيل

Patient No.: 4-300254

Accession No.: 4260532468

Age: 80Y

PATHOLOGY UNIT

Path. No. (7318-26).

Clinical Data & Nature of the specimen:

Left breast mass, Immunostaining.

Gross Examination:

Sections from paraffin block were treated with Dako monoclonal for ER, PR, Ki-67 antibodies. The sections were autostained in link 48 Dako autostainer using Envision Flex. And section treated by Ventana monoclonal antibody for HER-2/neu and stained on ultra penchmark. DAB detection kit & hematoxylin as counterstain.

Microscopic Examination:

Section treated for ER showed no nuclear staining of the tumor cells.

Section treated for PR showed moderate nuclear staining <10% of the tumor cells.

Section treated for HER-2/neu revealed intense complete membranous staining of the tumor cells.

Section treated for Ki-67 revealed nuclear staining of the about 25% of the tumor cells.

Diagnosis:

Left breast mass, Immunostaining:-

*ER: Negative (0/8).

*PR: Positive (4/8).

*HER-2/neu: Positive (score 3+).

*Ki-67 about 25 %.

Prof. Dr. Samia Gabal, MD



Pt. Branch: وحدات الرئيسي

Page 1 of 1

Reviewed By:

Printed by: 7123/اسماء احمد نوبى محمود



L A B S

Collection Date: 19/05/2026

Review Date: 23/05/2026

Referring Doctor: ا.د. ماره زكي

Patient Name: الاستاذة/ فريال علي اسماعيل

Patient No.: 4-300254

Accession No.: 4260531364

Age: 80Y

PATHOLOGY UNIT

Path. No.: (6998-26).

Clinical Data & Nature of the specimen:

Left breast UOQ mass, TCNB.

Gross Examination:

Four gray tan focally yellowish tissue cores measured up to 0.8 cm long, totally submitted.

Grossing by G. Amr.

Microscopic Examination:

*Histological type: Invasive duct carcinoma (NST).

*Histological grade: 2.

*Nottingham score: Tubular differentiation (3) + nuclear pleomorphism (2) + Mitosis (1).

*Insitu component: solid, intermediate grade with comedo necrosis.

*Lymphovascular emboli: No.

Diagnosis:

Left breast UOQ mass, TCNB:-

*Invasive duct carcinoma, NST, G2 (B5).

*For clinical & radiologic correlation.

N.B:-

Royal collage of pathologists breast lesions reporting categories include the following:-

B1. Normal tissue.

B2. Benign lesion.

B3. Lesion of uncertain malignant potential.

B4. Suspicious lesions.

B5. Malignant lesions.

Dr. Miran Atif, MD

Prof. Dr. Dalia M. Abouelfadl, MD



Reviewed By:

Pt. Branch: وحدات فرعية

Page 1 of 1

Printed by: 7283 نشات محمد نشات كامل



مستشفى مدينة نصر التخصصي مركز الأشعة والرنين المغناطيسي

أ.د. تامر أحمد كمال

أستاذ الأشعة التشخيصية
جامعة بنها

أ.د. مجدى إبراهيم بسيوني

أستاذ الأشعة التشخيصية
جامعة القاهرة

ULTRASOUND REPORT

Name:	Ferial Ali Ismael 176353
Date:	06 June, 2026
Ref. Dr. Proff.	

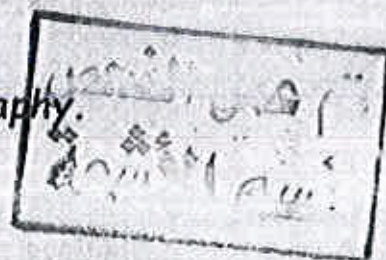
ABDOMINAL & PELVIC ULTRASONOGRAPHY:

- Liver is of normal size (14.2 cm), its parenchyma presents homogenous echopattern with no focal lesions .
- PV , CBD & intrahepatic biliary radicles are not dilated.
- Gall bladder is of average size with normal wall thickness & free from calculi or biliary mud..
- Spleen is of average size (10.3 cm), of uniform echo pattern.
- The pancreas and para-aortic areas are sonographically free.
- Both kidneys are of average size, normal in position with average parenchymal thickness & echo pattern. No calculi or back pressure changes.
 - Right kidney =
 - Renal parenchyma =
 - Left kidney =
 - Renal parenchyma =
- Normal UB : under filling .
- No free or localized intraperitoneal or pelvic fluid collection.

OPINION:

- Unremarkable pelvi-abdominal sonography.

MUCH OBLIGED,,
Dr. Azza Khalil



Patient's Name: Ferial Ali Ismail
Date: 2026-07-12
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

MULTISLICE NON CONTRAST CT SCAN OF THE CHEST:

TECHNIQUE: Multislice axial CT images of the chest with multiplanar reconstruction. No IV contrast was used.

FINDINGS:

- Left breast is out of field but showing ill-defined breast mass lesion
- Bilateral multiple scattered solid pulmonary nodules, largest one seen at lingual measuring about 2,5dm
- Right mild pleural effusion with underlying right lower lobar partial compression collapse.
- No pathologically enlarged hilar or mediastinal lymph nodes.
- No pericardial collection.
- CT signs are suggestive of cardiomegaly
- Exaggerated dorsal kyphosis with advanced dorsal spondylosis.
- Upper abdominal cuts are unremarkable.

Opinion:-

- Bilateral multiple scattered solid pulmonary nodules likely metatstatic
- Right mild pleural effusion with underlying right lower lobar partial compression collapse.
- Mild cardiomegaly

Dr.Nourhan Ewis

Name : Feryal Ali Ismail Ali

Date : 10-07-2026 08:24

Branch : Dokki

Medical No. : 413059

Ref.Physician Prof : Goustav Roussey Egypt

Tc^{99m} MDP SKELETAL SCINTIGRAPHY

History

88 year-old female patient , presented with history of left breast cancer and referred for metastatic work up .

Technique

- Anterior and posterior whole body scans were acquired 2 hours post injection of 20 mci of the tracer.

Findings

- Focal active tracer uptake is noticed opposite the anterior end of right lower ribs.
- Exaggerated degenerative changes are noticed at different levels of the spine.
- Osteoarthritic features are noted around big joints.
- Otherwise rest of the surveyed skeleton is unremarkable.

IMPRESSION

- Benign looking active osseous lesions at the anterior end of right lower ribs, likely traumatic for CT correlation.
- Rest of the scanned skeleton is unremarkable.

Much obliged
Dr. Alaa Nagy, MD
 Nuclear medicine

 R
E
T
E
N
E
C
E
I
C
T
I
O
N
A
G
I
A
D
I
L
E